

Special Issues: Suicide and Violent Patients

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Reinvent the Future.

Session Objectives

- 1 Students will know the risk and protective factors for suicide.
- 2 Students will be able to apply the characteristics of the suicidal patient to clinical scenarios.
- 3 Students will learn the components of a suicide and dangerous behavior assessment.
- 4 Students will understand the privilege of confidentiality and its exceptions.
- 5 Students will learn the best practices to safely assess the violent, aggressive patient

Why is suicide a public health problem?

Suicide is a significant problem in the United States

1. Suicide is the 11th leading cause of death in the US
2. In 2022, 49,476.00 Americans died by suicide
3. In 2022, there were an estimated 1,600,000 suicide attempts
4. In 2015, suicide and self-injury cost the US \$69 Billion

Suicide U.S.A.

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Additional Facts About Suicide in the US

- The age-adjusted suicided rate in 2022 was **14.21 per 100,000** individuals
- In 2022 men died by suicide 3.88x more than women
- On average, there are **132** suicide per day
- White males accounted for **68.46%** of suicide deaths in 2022
- In 2022, firearms accounted for **54.64%** of all suicide deaths

Suicide Data: United States

Suicide is a public health problem and leading cause of death in the United States. Suicide can also be prevented – more investment in suicide prevention, education, and research will prevent the untimely deaths of thousands of Americans each year. Unless otherwise noted, this fact sheet reports 2022 data from the CDC, the most current verified data available at time of publication (May 2024).

49,476 Americans died by suicide making it the **11th leading cause of death**.



- **3rd** leading cause of death for ages 15-24
- **2nd** leading cause of death for ages 25-34
- **4th** leading cause of death for ages 35-44
- **Over one third** of people who died by suicide were 55 or older



4.8% of adult Americans have thought about suicide.

1.6 million Americans attempted suicide.

54% of Americans have been affected by suicide in some way.

Men died by suicide **3.85 times** more often than females.

Women were **1.3 times** more likely to attempt suicide.

56% of firearm deaths were suicides.

55% of all suicides were by firearms.

In 2021, the suicide rate was **2 times higher for Veterans** than for non-Veteran adults over the age of 18.



90% of those who died by suicide had a diagnosable mental health condition at the time of their death.

46% of Americans ages 18+ living with a mental health condition received treatment in the past year.

73% of communities in the United States did not have enough mental health providers to serve residents in 2022, according to federal guidelines.

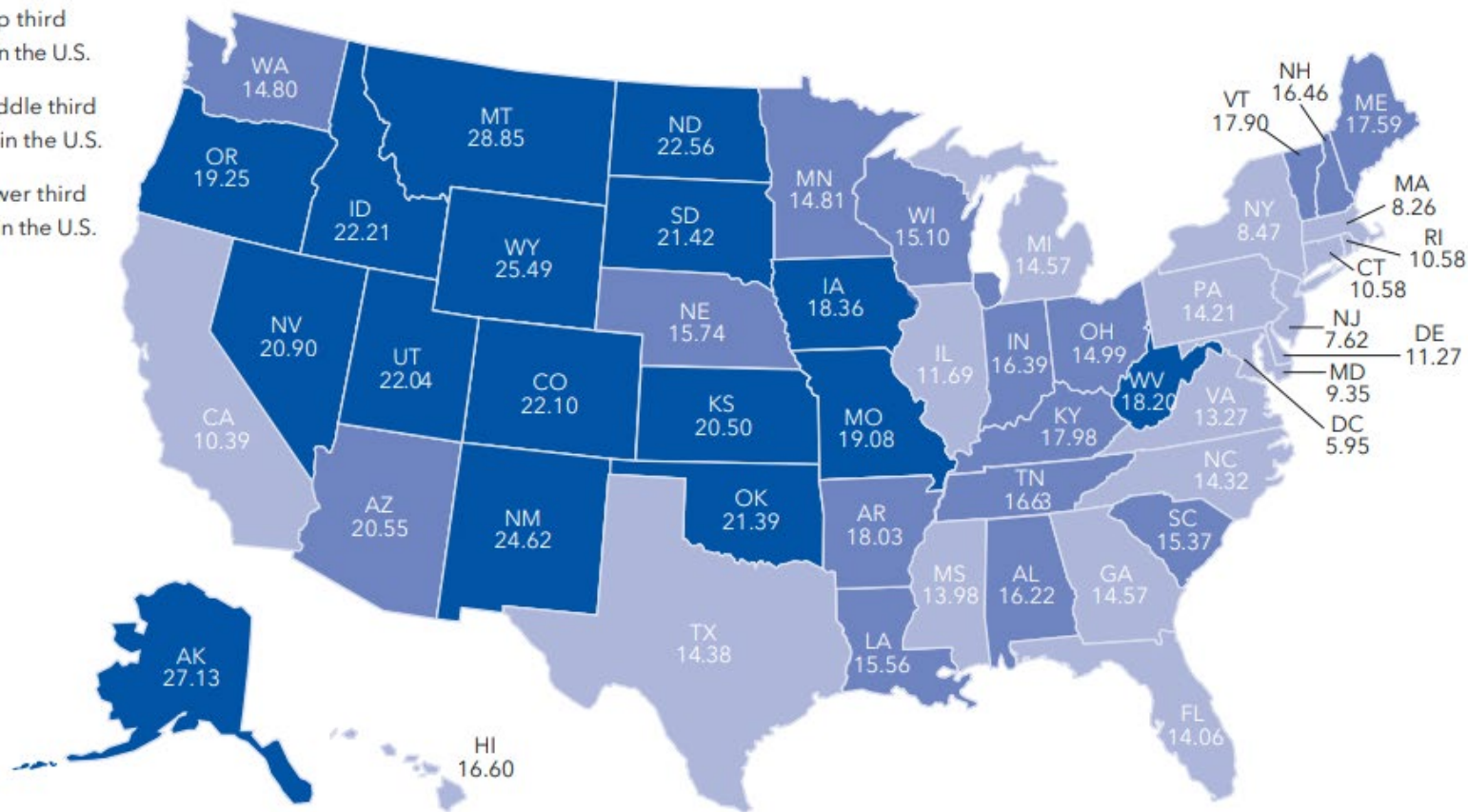


See full list of citations at afsp.org/statistics.



Suicide Data: United States

- States in the top third of suicide rates in the U.S.
- States in the middle third of suicide rates in the U.S.
- States in the lower third of suicide rates in the U.S.



See full list of citations at afsp.org/statistics.



10 Leading Causes of Death, United States

2022, All Deaths with drilldown to ICD codes, Both Sexes, All Races, All Ethnicities



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Ten Leading Causes of Death By Age Group, United States, 2022. CENTERS FOR DISEASE CONTROL AND PREVENTION, NATIONAL CENTER FOR INJURY PREVENTION AND CONTROL. WEB-BASED INJURY STATISTICS QUERY AND REPORTING SYSTEM (WISQARS) [ONLINE] AVAILABLE FROM URL: WWW.WISQARS.CDC.GOV

Suicide Data: Arkansas



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11th leading cause of death in Arkansas

3rd leading

cause of death for ages 10-24

2nd leading

cause of death for ages 25-34

5th leading

cause of death for ages 35-44

8th leading

cause of death for ages 45-54

11th leading

cause of death for ages 55-64

18th leading

cause of death for ages 65+

Suicide Death Rates

	Number of Deaths by Suicide	Rate per 100,000 Population	State Rank
Arkansas	618	20.61	11
Nationally	48,183	14.04	

See full list of citations at afsp.org/statistics.

66% of communities did not have enough mental health providers to serve residents in 2023, according to federal guidelines.

Over **three times** as many people died by suicide than in alcohol related motor vehicle accidents.

The total deaths to suicide reflected a total of **12,812 years** of potential life lost (YPLL) before age 65.

56% of firearm deaths were suicides.

63% of all suicides were by firearms.

Suicide Data: New York



Suicide is a public health problem and leading cause of death in the United States. Suicide can also be prevented – more investment in suicide prevention, education, and research will prevent the untimely deaths of thousands of Americans each year. Unless otherwise noted, this fact sheet reports 2021 data from the CDC, the most current verified data available at time of publication (January 2024).

15th leading cause of death in New York

3rd leading

cause of death for ages 10-24

2nd leading

cause of death for ages 25-34

5th leading

cause of death for ages 35-44

6th leading

cause of death for ages 45-54

11th leading

cause of death for ages 55-64

19th leading

cause of death for ages 65+

Suicide Death Rates

	Number of Deaths by Suicide	Rate per 100,000 Population	State Rank
New York	1,660	7.92	49
Nationally	48,183	14.04	

See full list of citations at afsp.org/statistics.

49% of communities did not have enough mental health providers to serve residents in 2023, according to federal guidelines.

Over **four times** as many people died by suicide than in alcohol related motor vehicle accidents.

The total deaths to suicide reflected a total of **32,406 years** of potential life lost (YPLL) before age 65.

41% of firearm deaths were suicides.

26% of all suicides were by firearms.

THE SUICIDAL PATIENT

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Demographic characteristics

- White
- Male
- Older age (> 55 years) middle age
- Divorced, never married, or widowed
- Unemployed
- Age 15-34 years (young)

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Patient history characteristics

- Previous suicide attempts, especially with serious intent, lethal means, or disappointment about survival
- Family history of suicide
- Victim of physical or sexual abuse

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Psychiatric characteristics

- Diagnosis: affective disorder, alcoholism, panic disorder, psychotic disorders, conduct disorder, severe personality disorder (especially antisocial and borderline).
- Symptoms: suicidal or homicidal ideation; depression, especially with hopelessness, helplessness, anhedonia, delusions, agitation; mixed mania and depression; psychotic symptoms, including command hallucinations and persecutory delusions.
- Current use of alcohol or illicit drugs.
- Recent psychiatric hospitalization.

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Environmental characteristics

- Recent loss (e.g., of a spouse or job)
- Social isolation
- Access to guns or other lethal weapons
- Social acceptance of suicide

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Medical characteristics

- Severe medical illness, especially with loss of functioning or intractable pain
- Delirium or confusion caused by central nervous system dysfunction.

Behavioral characteristics

- Antisocial acts
- Poor impulse control, risk taking, and aggressiveness
- Preparing for death (e.g., making a will, giving away possessions, stockpiling lethal medication)
- Well-developed, detailed suicide plan
- Statements of intent to inflict harm on self or others

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- All potentially fatal threats, gestures, and attempts should be taken seriously.
- Be aware of the possibility. If you don't consider the diagnosis, you won't make the diagnosis.
- Be empathic, try to establish a rapport before honing in on the issue of suicide.
- Perform a mental status examination.
- Ask about suicidal thoughts (e.g., ideas, wishes, motives).
- Ask about suicidal intent

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- Ask about suicide plans. Is there a detailed plan? Are the means available? Are the means lethal? Is there a chance for rescue?
- Are there any realistic plans for the future?
- Why now? Is there a precipitant? Anger? Loss? Helplessness? A social network?
- Information may be difficult to obtain if privacy not assured. However, information from friends or relatives may be essential.
- Review for the presence of risk factors (especially depression, alcoholism, psychosis, medical illness, and a history of prior attempts.)

Columbia-Suicide Severity Rating Scale

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COLUMBIA-SUICIDE SEVERITY RATING SCALE Screen Version

SUICIDE IDEATION DEFINITIONS AND PROMPTS	Past month	
	YES	NO
Ask questions that are bolded and <u>underlined</u>.		
Ask Questions 1 and 2		
1) Wish to be Dead: Person endorses thoughts about a wish to be dead or not alive anymore, or wish to fall asleep and not wake up. <u>Have you wished you were dead or wished you could go to sleep and not wake up?</u>		
2) Suicidal Thoughts: General non-specific thoughts of wanting to end one's life/commit suicide, "I've thought about killing myself" without general thoughts of ways to kill oneself/associated methods, intent, or plan. <u>Have you actually had any thoughts of killing yourself?</u>		
If YES to 2, ask questions 3, 4, 5, and 6. If NO to 2, go directly to question 6.		
3) Suicidal Thoughts with Method (without Specific Plan or Intent to Act): Person endorses thoughts of suicide and has thought of a least one method during the assessment period. This is different than a specific plan with time, place or method details worked out. "I thought about taking an overdose but I never made a specific plan as to when where or how I would actually do it....and I would never go through with it." <u>Have you been thinking about how you might kill yourself?</u>		
4) Suicidal Intent (without Specific Plan): Active suicidal thoughts of killing oneself and patient reports having <u>some intent to act on such thoughts</u> , as opposed to "I have the thoughts but I definitely will not do anything about them." <u>Have you had these thoughts and had some intention of acting on them?</u>		
5) Suicide Intent with Specific Plan: Thoughts of killing oneself with details of plan fully or partially worked out and person has some intent to carry it out. <u>Have you started to work out or worked out the details of how to kill yourself? Do you intend to carry out this plan?</u>		
6) Suicide Behavior Question: <u>Have you ever done anything, started to do anything, or prepared to do anything to end your life?</u> Examples: Collected pills, obtained a gun, gave away valuables, wrote a will or suicide note, took out pills but didn't swallow any, held a gun but changed your mind or it was grabbed from your hand, went to the roof but didn't jump; or actually took pills, tried to shoot yourself, cut yourself, tried to hang yourself, etc. If YES, ask: <u>How long ago did you do any of these?</u> • Over a year ago? • Between three months and a year ago? • Within the last three months?		

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SAFETY PLAN

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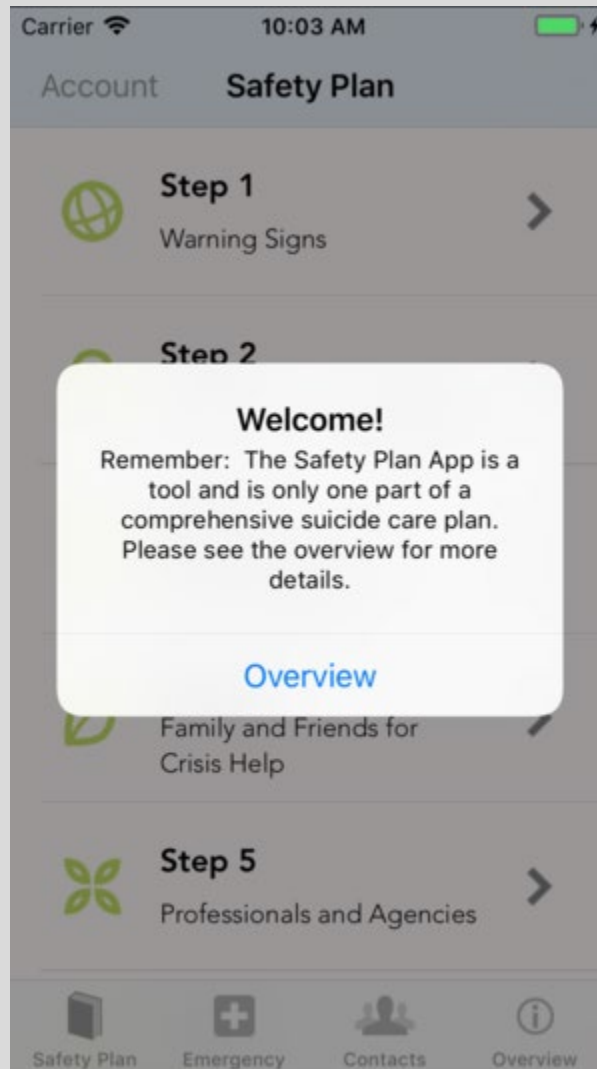
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Patient Safety Plan Template	
Step 1: Warning signs (thoughts, images, mood, situation, behavior) that a crisis may be developing:	
1.	_____
2.	_____
3.	_____
Step 2: Internal coping strategies – Things I can do to take my mind off my problems without contacting another person (relaxation technique, physical activity):	
1.	_____
2.	_____
3.	_____
Step 3: People and social settings that provide distraction:	
1. Name _____	Phone _____
2. Name _____	Phone _____
3. Place _____	4. Place _____
Step 4: People whom I can ask for help:	
1. Name _____	Phone _____
2. Name _____	Phone _____
3. Name _____	Phone _____
Step 5: Professionals or agencies I can contact during a crisis:	
1. Clinician Name _____	Phone _____
Clinician Pager or Emergency Contact # _____	
2. Clinician Name _____	Phone _____
Clinician Pager or Emergency Contact # _____	
3. Local Urgent Care Services _____	
Urgent Care Services Address _____	
Urgent Care Services Phone _____	
4. Suicide Prevention Lifeline Phone: 1-800-273-TALK (8255)	
Step 6: Making the environment safe:	
1.	_____
2.	_____
Safety Plan Template 5/2008 Barbara Stanley and Gregory K. Brown, is reprinted with the express permission of the authors. No portion of this Safety Plan Template may be reproduced without their express, written permission. You can contact the authors at bbs52@cornell.edu or gkbrown@gmail.com at nyinstitute.edu.	
The one thing that is most important to me and worth living for is:	

The Safety Plan Application

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Source: Stanley-Brown Safety Plan on the App

'Sad Personas'

- S- sex
- A - age
- D - depression

'Sad Personas' (cont.)

- P – previous attempt
- E – ethanol abuse
- R – rational thinking loss
- S – social supports lacking
- O – organized plan
- N – no spouse
- A – availability of lethal means
- S – sickness

NO HOPE SCALE

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No framework for meaning
Overt change in clinical condition

Hostile Interpersonal Environment
Out of hospital recently
Predisposing personality factors

Excuses for dying are present and
strongly believed

PSYCHOPHARMACOLOGY AND SUICIDE

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CLOZAPINE **reduces risk** FDA-Schizophrenia 2023

LITHIUM **reduces risk** 5 fold decrease risk in bipolar disorder

ANTIDEPRESSANTS
SSR's may increase risk in children and adolescent
use but monitor

Ketamine
helpful in refractory MDD
IV Infusions-anti suicidal effect
FDA approval intranasal preparation



Legal Issues in Psychiatry

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Five Components of Dangerousness



The Five Components of Dangerousness are:

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1. ***Nature of harm or conduct:*** Does the individual threaten bodily harm, or is it harm to property, or individual sensitivities?
2. ***Magnitude of harm:*** Does the individual threaten murder or is the threat a punch in the nose?
3. ***Probability:*** How likely is it that the feared harm will occur?
4. ***Imminence:*** When will the threatened action occur?
5. ***Frequency:*** How frequently will the threatened action occur?



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Confidentiality



Confidentiality

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1. DEFINITION

In the American Psychiatric Association's Principles of Medical Ethics with Annotations Especially Applicable to Psychiatry, Annotation 1 reads, *"Confidentiality is essential to psychiatric treatment. This is based in part on the special nature of psychiatric therapy as well as on the traditional ethical relationship between physician and patient."*

Confidentially is defined mnemonically as the clinician's obligation to keep information learned in that relationship unavailable to third parties.



Confidentiality *(cont'd)*

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2. Major Exceptions

- Patient requests release of information
- Duty to protect
- Emergencies
- Mandatory reporting statutes
- Court-ordered evaluations
- Patient initiates litigation



Confidentiality (*cont'd*)

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3. Circle of Confidentiality



Patient's Family
Lawyer
Outside Therapist
Previous Therapist

Police



Confidentiality (*cont'd*)

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4. Principles to follow when breaching confidentiality is necessary

- Notify patient whenever possible
- Utilize therapeutic alliance
- Encourage patient to inform (e.g., duty to protect)
- Document rationale for action



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Tarasoff
Case

Violence in Society

- Culture
- Race
- Poverty
- Firearms
- Drugs
- Economic Crimes
- Non-Economic Crimes

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Psychiatric Disorders

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- Antisocial personality
- Paranoid personality
- Borderline personality
- Narcissistic personality
- Histrionic personality
- Other personality disorders
- Intermittent explosive disorder
- Schizophrenia
- Mania
- Depression
- Anxiety Disorders
- Mental retardation
- Other disorders

Common Etiologies of organically induced aggression

1. Traumatic brain injury
2. Stroke and other cerebrovascular disease
3. Medications, alcohol and other abused substances, over-the-counter drugs
4. Delirium (e.g., hypoxia, electrolyte imbalance, anesthesia and surgery, uremia)
5. Alzheimer's disease

Common Etiologies of organically induced aggression

- Chronic neurological disorders: Huntington's disease, Wilson's disease, Parkinson's disease, multiple sclerosis, systemic lupus erythematosus
- Brain tumors
- Infectious diseases(encephalitis, meningitis, AIDS)
- Epilepsy (ictal, postictal, and interictal)
- Metabolic disorders: hyperthyroidism or hypothyroidism, hypoglycemia, vitamin deficiencies porphyria

Medications and drugs associated with aggression

- Alcohol – intoxication and withdrawal states
- Hypnotic and anti-anxiety agents (e.g., barbituates, benzodiazepines): intoxication and withdrawal states
- Analgesics – opiates and other narcotics – intoxication and withdrawal states

Medications and drugs associated with aggression

- Steroids (prednisone, cortisone, and anabolic steroids)
- Antidepressants – especially in initial phases of treatment
- Amphetamines and cocaine – aggression associated with manic excitement in early stages of abuse and secondary to paranoid ideation in later stages of use

Medications and drugs associated with aggression

- Antipsychotics – high potency agents that lead to akathisia
- Anticholinergic drugs (including over-the-counter sedatives) associated with delirium and central anticholinergic syndrome

INCREASED RISK OF ATTACKS

- Younger Clinicians
- Inexperienced Clinicians
- Inpatient and Emergency Rooms
- Outpatient More Likely to Involve Weapons
- Setting Limits for Patients
- Provocative or Aggressive Clinicians
- Psychotic Disorder with command hallucinations or fixed (persecutory) delusions
- Personality Disorders

DEALING WITH THE ACUTE PATIENT

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- Safety First
- Setting of Interview
- Emotional Reactions
- Physical Environment-(door, objects, furniture, etc)
- Dress
- Physical Defense
- Talking with the Patient
- Weapons

PREVENTION

- Ward Milieu
- Adequate Staff
- Trained Staff
- Recognize Warning Signs
- Staff Knowledge of Their Feelings

Are People with Mental Illness Less Likely to be Violent than the General Population?

- Review of police reports and arrest records would seem to support this
- But many violent acts never reach the attention of the police
- Charges are not brought because the person is ill.
- Around one-third of all psychiatric admissions are preceded by violence.

Prediction of Violence

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- Demographic – Male, young (15-24) poor, uneducated, no supportive social network.
- Diagnostic – Neuropsychiatric brain syndrome (including intoxications, TBI) psychosis, personality disorders.
- Past History – Violence, early victimization
- Psychological – low frustration tolerance, low self-esteem, low tolerance for interpersonal closeness, low tolerance for criticism, externalization.

History of present situation

- Has anyone been harmed?
- Thoughts of harming someone – a particular person or people
- Plans to harm – have preparations been made, weapons in possession, access to weapons in possession, access to weapons.
- Under what conditions would the patient feel impelled to act?
- Current alcohol or drug use, behavior when intoxicated
- Current anxiety and depression
- Current quarrels, jealousy, provocations
- Driving habits – reckless or aggressive

Past History

- Previous violent thoughts, plans, or acts; against whom and why; why didn't the patient act at that time, if plan wasn't carried out; premeditated or impulsive; punishments or consequences
- Arrest, prison
- Fighting, reckless driving, vandalism
- Sanctioned violence – military police
- Head trauma, psychosis, seizures, encephalitis, meningitis, menstrual history and symptoms
- Victim of violence
- Family history of violence

Childhood

- Parental brutality or seduction
- Degree to which violence was normative in family and culture of origin
- Fire setting, bed-wetting, cruelty to animals

Physically Approaching the Patient

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- Body posture, Facial expression, voice tone are still understood by patients impaired by delirium, psychosis, anger or fear. They cannot process and understand your words.
- Allow more physical space than usual. Buffer zone is 4x larger in violent than in non-violent persons. Greater to the rear than to the front, a reversal of the pattern in non-violent people. Therefore, do not approach patients from behind; however, do not face them directly because this can be perceived as confrontational.

Physically Approaching the Patient

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If you must retreat rapidly:

1. Do not run => Easy to grab from behind
2. Do not run backward => Easy to knock over
3. Turn sideways => present smaller body profile as a target
4. Keep your eyes on the assailant. Arms in front of your face and neck. Do not close fists.
5. Move away by moving the nearest foot across behind your other foot and then moving the front foot across in front away from your assailant and repeat the pattern.
6. Ideally you and patient should be equal distance from the door so the patient will not feel “cornered”

DO NOT BLOCK THE DOOR OR EXIT

Physically Approaching the Patient (contd.)

- Be careful about direct eye contact
- Keep your facial expressions neutral, accepting, and attentive.
- Keep your hands to the midline of your body where they will be more ready to ward off a blow, => DO NOT fold them akimbo or at your waist <= Confrontational
- Avoid easy striking range. At least 1 ½ arms length away so patient will have to move toward you to strike.
- =>Never turn your back and stay near the door or an escape route.
- Attitude toward punishment or other negative consequences
- Degree to which violence is normal in current social circle
- Suicidal thoughts or plans

Safe and Supportive Stance

1. Safest shoulder side
2. Distancing
3. Front foot at 12:00 position; back foot at 10:00 or 2:00
4. Blading
5. Knees slightly bent
6. Hands above belt line

12:00



2:00



Verbally Approaching the Patient

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- Interview in a calm, direct reassuring manner
- Refer to patients by surname with title
- Explain your own behavior
- Keep questions, requests and comments very simple, brief, and repeat information to patient several times.
- Constantly clarify that talking and feeling are acceptable that acting violent is not.
- Make it clear patient will be held responsible for his or her actions.
- Acknowledge patient's ability to harm others, then establish your position that this would not be acceptable and that the staff will act to prevent harm to the patient or to others.
- Do not let this sound like a threat but state it as a matter of fact and balance it with a statement that you will do what you can to help.

Summary Statements

- *Knowing the risk factors for suicide and dangerous behavior including violence can help save lives.*
- *Risk Assessment is a skill that must be learned and practiced. Psychiatrist cannot predict but can carefully utilize risk and protective factors to help prevent dangerous behavior, violence and suicide.*
- *Psychiatric patients have rights to confidentiality and making their own health care decisions, including the right refuse medication.*
- *Psychiatric patients are assumed to have capacity to make decisions until proven otherwise through the court system and due process.*

Lecture Feedback Form:

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YXREL4R](https://comresearchdata.nyit.edu/redcap/surveys/?s=HRCY448FWYXREL4R)